NCCP 00214 · VERSION 8

Your treatment, explained

First-line non-squamous NSCLC · bevacizumab component only · NCCP 00214 v8 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Bevacizumab 7.5 · lung

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment stages
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

Three medicines. Different jobs.

To help control advanced non-squamous lung cancer alongside the prescribed chemotherapy, then possibly as continuation treatment.

ANTI-VEGF ANTIBODY

Bevacizumab

Bevacizumab



Blocks the VEGF growth signal

Bevacizumab binds VEGF, a signal that helps new blood vessels grow. It can slow tumour growth but also affect normal blood vessels and wound healing.

Blocks the VEGF growth signal → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Report bleeding, severe tummy pain, unusual headache or vision changes urgently. Tell the team about any surgery or wound.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Treatment → review → next steps

WITH CHEMOTHERAPY

With chemotherapy

1–6

Up to 6 three-week cycles with the named platinum regimen. Bring its schedule and safety plan too.

POSSIBLE CONTINUATION

Possible continuation

Review

Bevacizumab alone every 3 weeks if the team confirms ongoing benefit and acceptable side effects. No fixed total number of cycles.

Your team confirms each stage and discusses any changes with you.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

Check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment	Confirm lung indication / partner	Bevacizumab Day 1	Follow partner schedule	Up to 6 combined cycles; review	Continuation only if agreed
Bloods / urine	FBC, kidney/liver, urine protein	Regular blood/urine checks	Quantify protein if needed	Review trends	Follow abnormalities
Blood pressure / heart	BP and cardiac history/ exam	BP before each cycle	Extra BP if raised	Review control	BP post-treatment
Scans / other tests	Individual disease plan	No fixed scan interval	INR if warfarin	Response review	Follow-up agreed

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00214 v8 · official regimen · Avastin current EMA product information

THE WHOLE TREATMENT JOURNEY

Bevacizumab 7.5 · lung: treatment and checks

First-line non-squamous NSCLC · bevacizumab component only

Bevacizumab is given by drip on Day 1 every 21 days. In this lung setting it starts with platinum chemotherapy for up to 6 cycles, then may continue alone while helping and tolerable. Your team must supply the partner chemotherapy schedule.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00214 · SMPC	Confirm lung indication / partner	Bevacizumab Day 1	Follow partner schedule	Up to 6 combined cycles; review	Continuation only if agreed
Bloods / urine NCCP 00214 · SMPC	FBC, kidney/liver, urine protein	Regular blood/urine checks	Quantify protein if needed	Review trends	Follow abnormalities
Blood pressure / heart NCCP 00214 · SMPC	BP and cardiac history/ exam	BP before each cycle	Extra BP if raised	Review control	BP post-treatment
Scans / other tests NCCP 00214 · SMPC	Individual disease plan	No fixed scan interval	INR if warfarin	Response review	Follow-up agreed

What each check is for

1

Prepare

Before treatment

To help control advanced non-squamous lung cancer alongside the prescribed chemotherapy, then possibly as continuation treatment.

Ask / act: Confirm the aim, phase, prescription, tests and contacts.

Sources: NCCP 00214

2

Review

Before each cycle / at agreed reviews

Check benefit and tolerability

Ask / act: Report symptoms and confirm the next plan.

Sources: NCCP 00214

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.30

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

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Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Bevacizumab infusion Day 1 every 21 days after review.	Day 1																				
Partner chemotherapy plan	See the named chemotherapy schedule; additional treatment visits may occur.																				
Symptom awareness	Bleeding, headache, tummy pain and new symptoms can occur at any time.																				

Symptoms can occur at any time: Side effects and infection do not follow a guaranteed day-by-day risk curve. Contact the team when symptoms occur.

Changes for safety: Treatment may be delayed, reduced or stopped. Do not make up missed doses or change your tablets yourself.

> **Why these bands stay broad:** Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Read the whole cycle as text

Day 1: Bevacizumab infusion

After blood pressure, urine and other safety checks. Partner chemotherapy follows its own schedule. Use the current prescription and contact plan.

Day 2–21: Between bevacizumab doses

Watch symptoms, take only prescribed support medicines and follow the separate chemotherapy plan. Use the current prescription and contact plan.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Partner chemotherapy

This covers bevacizumab only. Bring the separate chemotherapy schedule, anti-sickness plan and urgent-contact instructions.



STEP 2

Blood pressure and urine

These checks can find problems before you feel unwell. Take blood-pressure medicines as prescribed; do not adjust them yourself.



STEP 3

Surgery and dental procedures

Tell oncology before any operation or invasive procedure. The team must agree treatment pauses and assess wound healing.



STEP 4

Support medicines

Bevacizumab alone does not usually need routine anti-sickness prevention. Chemotherapy partners may need a different plan.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00214 v8 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Partner chemotherapy

This covers bevacizumab only. Bring the separate chemotherapy schedule, anti-sickness plan and urgent-contact instructions.

2 Blood pressure and urine

These checks can find problems before you feel unwell. Take blood-pressure medicines as prescribed; do not adjust them yourself.

3 Surgery and dental procedures

Tell oncology before any operation or invasive procedure. The team must agree treatment pauses and assess wound healing.

4 Support medicines

Bevacizumab alone does not usually need routine anti-sickness prevention. Chemotherapy partners may need a different plan.

Sources: nccp · ncis · antiemetic

Understanding the evidence

AVAiL showed longer progression-free survival with bevacizumab added to the studied chemotherapy, without a significant overall-survival improvement.

Direct evidence for 7.5 mg/kg with the studied cisplatin/gemcitabine backbone; outcomes must not be borrowed from 15 mg/kg carboplatin/paclitaxel trials or colorectal studies.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00214 v8. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00214 v8 · official regimen
- Avastin current EMA product information
- NCCP antiemetic classification v 6 and oral pathway
- NCIS medical oncology antiemetic medicines

- AVAiL final analysis

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6554/214_Bevacizumab_7.5.pdf

https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/20150572/